IDAHO RURAL HEALTH TRANSFORMATION

Healthcare Infrastructure Support (HIS) Subgrant Frequently Asked Questions

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1. Applicant Eligibility & Organizational Status

FAQ: Who is eligible to apply for HIS funding?

Applicants should determine eligibility by reviewing Sections 2.1–2.3 of the posted solicitation. IDHW will not make a binding, entity-specific eligibility determination through the Q&A register. An applicant must fall within an eligible applicant type and satisfy all applicable geographic/service, SAM.gov/UEI, minimum qualification, and exclusion requirements. Applicants should explain in their application how they meet those requirements.

FAQ: Can a newly formed organization apply if it does not have prior federal grant experience or a Single Audit history?

Lack of prior federal award experience or Single Audit history does not, by itself, necessarily make an otherwise eligible organization ineligible to apply. Applicants should complete the required financial capacity and risk-assessment information based on their actual organizational circumstances and indicate when a requested item is not applicable because the organization does not have prior federal award history.

IDHW will evaluate organizational capacity and pre-award risk based on the information submitted and may request additional documentation or impose additional award conditions when appropriate.

2. Rural Geographic & Multi-Location Eligibility

FAQ: Can an organization headquartered outside an eligible rural area apply or operate an urban hub?

An applicant may have an administrative or headquarters location outside an eligible rural area, but the funded project, costs, and beneficiaries must satisfy the solicitation's rural geographic/service requirements. Applicants should clearly identify each project location, the eligible rural population or facilities served, and how costs are allocated to the rural benefit.

3. Multiple Applications, Categories & Project Structuring

FAQ: How should applicants structure multiple projects, categories, or locations?

The Department prefers one application package from an eligible applicant where multiple categories or locations are part of one integrated project. They may be presented together if each component, location, funding category, budget amount, need, and deliverable is clearly distinguishable. However, unrelated projects should not be combined in a way that prevents independent scoring or budget review. Applicants should follow the posted submission requirements for the final application package or packages.

FAQ: Does the non-duplication requirement prevent an applicant from expanding an existing service?

Not necessarily. RHTP funding may support new activities or an expansion of existing activities when the proposed funding does not replace or duplicate funding already available for the same activity.

For example, an applicant may propose to expand service hours, geographic reach, capacity, technology, or access where the application clearly identifies the additional need being addressed and demonstrates that RHTP funds would support the new or expanded activity rather than replace an existing funding source.

Applicants should clearly describe existing services and funding and explain what is new, expanded, or otherwise different about the proposed RHTP-funded activity.

4. Needs Assessments — Requirements, Sources & Timing

FAQ: Does the applicant have to conduct its own needs assessment?

No. The assessment does not need to have been completed by the applicant. Applicants may use a relevant needs assessment completed by another organization or entity. Examples may include a Community Health Needs Assessment (CHNA) completed by a hospital or Critical Access Hospital, a public health district needs assessment, or another credible community or health needs assessment.

The application must clearly identify the specific need documented in the assessment and explain how that need relates to the population the proposed application will serve.

The population addressed in the needs assessment should reasonably align with the population addressed in the application. For example, an applicant serving residents of a particular rural county may rely on a hospital, public health district, or other community needs assessment that evaluates that same county or population and documents the need the applicant proposes to address.

Applicants should clearly cite or reference the relevant portion of the assessment so reviewers can understand where the identified need is documented.

FAQ: What is considered an “internal” needs assessment?

An internal needs assessment may be completed by the applicant organization and does not have to be performed by an outside consultant or third party. Examples might include a documented business or strategic plan, formal environmental scan or SWOT (Strengths, Weaknesses, Opportunities, Threats) assessment. The assessment must still provide reasonable documentation of the identified need, apply to the population the proposed project will serve, and meet the other needs-assessment requirements in the solicitation.

An internal assessment may draw on information from organizational leadership, clinical or operational staff, utilization data, service gaps, resource assessments, patient or community information, or other relevant sources. Applicants should clearly document how the assessment identified the need addressed by the proposed project.

FAQ: What kinds of needs assessments may be used?

A needs assessment may come from the applicant or from another credible organization. Examples may include:

- a hospital or Critical Access Hospital Community Health Needs Assessment (CHNA);
- a public health district or local health department assessment;
- a community health or regional needs assessment;
- a health plan or other healthcare organization's needs assessment; or
- another credible assessment that documents the relevant need and population.

The important consideration is not who prepared the assessment, but whether it provides relevant documentation of the need the proposed project is intended to address and applies to the population the project will serve.

FAQ: What should the applicant include in the application when using another organization's needs assessment?

The applicant should clearly identify:

- the assessment being relied upon;
- the population or geographic area covered by the assessment;
- the specific need identified in the assessment that the application will address; and
- where that need can be found in the assessment, such as a page, section, table, or finding.

Applicants should not assume reviewers will make the connection between the assessment and the proposed project. The application should clearly explain that relationship.

FAQ: What if an applicant does not have a qualifying needs assessment and cannot rely on an assessment completed by another organization?

If an applicant does not have a qualifying needs assessment and cannot use an existing assessment from another organization to document the need addressed by its application, the applicant will need to obtain a qualifying needs assessment.

Applicants may arrange and fund their own assessment, or they may contact the Department regarding access to contracted needs-assessment support. Department-supported assessment capacity is still being established, and availability and completion timelines cannot be guaranteed.

Because of the limited timeline for this funding opportunity, organizations that still need a new needs assessment completed may not be able to obtain one in time to submit a qualifying application for this round of funding. Those organizations are encouraged to begin preparing now for future RHTP funding opportunities, including ensuring that an appropriate needs assessment is completed and available to support a future application.

5. Award Timing, Stage 2 Rules & Earliest Allowable Costs

FAQ: When can costs be incurred, and what is allowed during Stage 2?

Costs may not be incurred under the subgrant until the agreement is signed by both the awarded organization and the Department and any applicable Stage 2 approval conditions are satisfied. Costs incurred before the date of the Department's authorizing signature are the organization's responsibility and are not reimbursable under the subgrant. During Stage 2, applicants may continue non-chargeable planning activities, but Stage 2 revisions must remain within the scope of the competitively reviewed Stage 1 project.

6. Administrative Cost Cap, Indirect Costs & Personnel

FAQ: How does the 10% administrative cost cap apply to indirect costs and personnel?

RHTP administrative costs are subject to the 10% administrative cap, including both direct administrative costs and indirect costs treated as program administration. Personnel who directly carry out approved program activities may be allowable as direct program costs when necessary, reasonable, allocable, and properly documented. Personnel performing grant administration, accounting, general management, or similar administrative functions count toward the administrative cap. RHTP funds may not supplant or replace existing funding for activities, services, or costs already supported by another funding source. Subrecipients should ensure proposed personnel and administrative costs are new, expanded, or otherwise attributable to RHTP activities and are not simply shifting existing operating costs to the RHTP award. Applicants must use the indirect-cost method allowed under the solicitation and 2 CFR Part 200, but the RHTP 10% administrative limitation still applies. Temporary implementation or consulting staff should be budgeted according to the nature of the cost and clearly tied to the approved project. Applicants should explain the function of each position or contracted role so IDHW can determine the appropriate treatment.

7. Minor Renovation / Alteration & Capital Expenditure Rules

FAQ: What happens if an approved renovation project experiences delays and cannot be completed by September 30, 2027?

Applicants should propose projects that can reasonably be completed within the stated project period. If an awarded project later experiences unforeseen delays, the subrecipient must notify IDHW as soon as the delay is identified and before making material changes to the approved schedule or scope.

Any extension or modification would be subject to applicable RHTP requirements and IDHW approval and cannot be guaranteed. Applicants should therefore account for reasonably foreseeable permitting, procurement, equipment lead-time, contractor availability, and other implementation risks when developing the proposed project schedule.

FAQ: Are ADA upgrades, wall relocations, or space repurposing allowed?
Potentially yes if they qualify as minor and aligned with program purposes. Significant structural changes may be disallowed.

Below are CMS Guidelines regarding a minor renovation / alteration:
✓ Building & Equipment Repairs: Repairing existing structures and equipment to ensure patient and staff safety and comfort, including asbestos abatement.

✓ Minor Building Renovations: Repurposing existing buildings to offer new clinical services.

✓ Interior Modifications: Installing or relocating interior walls and partitions to create new patient rooms; installing soundproofing to offer telehealth services.

✓ Workspace Reconfiguration: Creating additional clinical or patient rooms, open office layouts or converting private offices into clinical space.

✓ Lighting & Electrical: Upgrading fixtures to energy-efficient systems for lower energy costs.

✓ HVAC & Plumbing: Replacing vents and thermostats for improved climate control and lower energy costs; replacing inefficient boilers.

✓ Accessibility Improvements: Installing automatic door openers and ADA-related upgrades, repaving and marking parking lots when necessary for patient access to care.

✓ Security & Safety: Installing or upgrading security cameras or access control systems.

✓ Childcare Center Renovations: Conducting renovations when critical to support approved workforce initiatives.

✓ Permanent Functional Signage: Installing directional instructions for patients and EMS vehicles.

✓ Workforce Housing Renovations: Carrying out renovations that support recruitment, training or retention of staff as part of an approved workforce initiative.

Limitations & Exclusions
× No expansion, additions, or increases in square footage that cause a significant and substantial rise in property value or extend useful life.

× Demolition Activities are considered construction or major renovation and are not allowed.

× Cosmetic Improvements are not allowed (painting, carpet upgrades, artwork, décor that enhances visual appeal without altering core structure).

× Construction (new builds, breaking ground) is entirely unallowable under this grant program.

× Category J funds must supplement, not replace, expenditures already planned or obligated through other funding sources.

8. Technology, Cybersecurity, EHR & AI

FAQ: Are technology, cybersecurity, AI, and EHR projects eligible?
Technology, interoperability, cybersecurity, AI tools, and eligible EHR upgrades may be allowable when they directly support the approved rural RHTP project, are new or expanded rather than duplicative, and meet the solicitation's specific technology limitations. The 5% EHR-replacement limitation applies to replacement of an existing HITECH-certified EHR as described in CMS guidance; integration, interoperability, or add-on functionality is not automatically an EHR replacement and will be evaluated based on the actual scope.

9. Procurement, Contracting & Budget Classification

FAQ: What procurement, contracting, and budget-classification requirements apply?
Subrecipients must follow the procurement standards applicable to them under 2 CFR 200.317–200.327, applicable State/local/tribal law, and their own documented procurement procedures. Applicants may obtain quotes and conduct planning before award, but pre-award activity does not authorize reimbursement or bypass competition requirements. The Stage 1 budget should identify each material item or service clearly enough for allowability and reasonableness review. If the procurement-category dropdown does not contain the needed option, applicants should type 'Consulting' when not seeking Cybersecurity Consulting and 'Practice Transformation' when applicable, consistent with the solicitation's allowable categories.

10. Application Templates & Page Limits

FAQ: What application templates, attachments, and page limits apply?
Applicants should use the required posted application templates. The Stage 1 Project Narrative limit is 10 pages as stated in Table 4.1. Appendix D is the required narrative template, and the title page is not counted in the 10-page limit. Letters of support may be submitted but are not a scored application component. Applicants should preserve the requested content and structure of required templates; minor

spacing/formatting adjustments that do not remove required fields or content should not change the substantive application requirements.

FAQ: Should the application budget include only the current funding year or all five years of the RHTP?

Applicants should complete the required budget based on the funding period covered by this solicitation. Information regarding future years may be included in the work plan where requested for planning purposes, but future RHTP funding is not guaranteed. Applicants should not include future-year funding as though it has already been awarded unless specifically instructed by the solicitation or application template.

11. Evaluation, Risk Assessment & Monitoring

FAQ: How will applications be evaluated and monitored?

Applications will be evaluated using the scoring criteria and process in Section 5 of the solicitation. Recommended applicants will also undergo the pre-award risk assessment described in Section 5.3; limited prior federal grant experience is a risk factor, not automatically a disqualifier. Post-award monitoring will be risk-based and may include review of financial and performance reports, desk review, corrective action, and site visits. Requests for Medicaid or other IDHW data remain subject to the Department's normal data-governance and request processes.

FAQ: What is included in a desk review?

A desk review is IDHW's routine subrecipient monitoring conducted remotely, typically on a monthly or quarterly basis. It includes review of subrecipient expenditures, progress toward program goals and objectives, and required monitoring documentation using standard tools and templates, as applicable.

12. Project Period & Multi-Year Planning

FAQ: How should applicants address multi-year planning and future-year funding?

Applicants may describe multi-year plans where requested by the solicitation, but future RHTP funding is contingent on CMS continuation funding, Department decisions, and future award actions. Applicants should not assume that the Year 1 award amount will remain stable in later years. Activities funded by this solicitation must be achievable within the applicable subaward period of performance.

13. Partial Funding, Line-Item Approval & Award Scaling

FAQ: Can IDHW reduce, condition, or partially fund an application?

Applicants should request the amount necessary to accomplish the proposed project and provide a clear basis for each cost. IDHW will fund only costs that are allowable, reasonable, supported by the approved scope, and within available funding. IDHW may reduce, condition, or partially fund an application.

14. Project Coordination

FAQ: Who should serve as the primary contact, and what project coordination should awardees expect?

The primary contact should be the individual authorized to coordinate the application and subaward with IDHW and able to obtain timely information from the project, fiscal, and leadership teams. Awardees should expect ongoing coordination with IDHW and designated RHTP support contractors as described in the solicitation. Specific meeting cadence, tools, and workflows will be provided after award as implementation processes are finalized.

15. Webinar Recording, Slides & Q&A Process

FAQ: Where will webinar materials and official Q&A responses be posted?

The webinar slide deck and recording are posted on the IDHW RHTP Funding Opportunities webpage. Questions submitted through the designated process will be reviewed and answered through the official Q&A/clarification process so that all potential applicants have access to the same information.

16. Single Audit Requirement

FAQ: When is a Single Audit required?

A Single Audit is required under 2 CFR 200.501 for a non-federal entity that expends $1,000,000 or more in federal awards during its fiscal year, subject to the rule's program-specific audit provisions. Applicants should submit their most recent Single Audit (if applicable). An entity below the threshold is exempt from the Federal Single Audit requirement for that year, but its records remain subject to review and IDHW may request other financial information for risk assessment.

17. Project-Specific Allowability — Example: Generator at a Retail WIC Site

FAQ: Could a generator at a retail WIC site be eligible under HIS?

Based on the solicitation's Healthcare Infrastructure Support purpose, the applicant must demonstrate that the requested generator is an eligible infrastructure/equipment cost that directly supports delivery of healthcare to eligible rural populations and is not a general retail operating expense or duplicative of another funding source. If the primary benefit is preservation of general retail food inventory, the request appears outside the intended HIS scope. Final eligibility is determined through application review.

18. Budget Template, Supplies & Category Mechanics

FAQ: How should applicants complete budget categories, supplies, and other template fields?

Applicants should itemize material costs sufficiently for IDHW to understand the

nature, quantity, estimated unit cost, and applicable funding category. If the workbook dropdown does not contain an expressly allowable category, manually type in the category label as applicable. Supplies and lower-cost items should be budgeted according to the template instructions and the organization's accounting treatment; do not confuse the federal equipment capitalization threshold with any separate IDHW property/inventory-reporting threshold. Applicants should preserve all required budget fields even if they adjust spacing or formatting.

19. Applicant Technical Assistance & Needs-Assessment Support

FAQ: What technical assistance or needs-assessment support will IDHW provide applicants?

IDHW will provide the published webinar/Q&A so that applicants receive consistent information. Where the Department has arranged contracted needs-assessment support, applicants should use the Department's designated request process. Assistance with obtaining or reviewing a needs assessment does not constitute a determination that an applicant or proposed project is eligible or will be funded, and applicants remain responsible for submitting a complete application by the deadline. Additional questions or requests for support can be made using the IdahoRHTP@dhw.idaho.gov email address.

20. HIS Scope — Infrastructure vs. Operating or Service-Delivery Costs

FAQ: How should applicants distinguish eligible HIS project costs from general operating or service-delivery costs?

This Healthcare Infrastructure Support solicitation is focused on the eligible categories and activities identified in the posted solicitation. Costs that are necessary to implement an approved infrastructure project may be considered when they fit an allowable budget category and are necessary, reasonable, allocable, and adequately justified. Applicants should not assume that general ongoing operating expenses, standalone service-delivery costs, or workforce activities outside the HIS scope are allowable merely because they support rural health. Staffing questions should also be evaluated under the RHTP administrative-cost rules and the specific HIS project scope. HIS funding cannot be used to duplicate or supplant (replace) existing services or funding. Work supported must be proven to be new or expanded through the “Non-Duplication Assessment” process.